4

Article 12 and Autonomy

1 INTRODUCTION

Chapter 2 described key concepts in disability and decision-making and how these have driven and informed the development of policy and legal frameworks from favouring paternalism to promoting autonomy. This trajectory is evidenced by the move from institutionalisation to guardianship to supported decision-making. It parallels the development of models of disability that shifted from the dominance of a charity model though to a medical, then social model. Chapter 2 also traced the development of the CRPD, with the dominant framing of Article 12 at the highpoint on a trajectory of rights development, signalling people with disability as subjects of human rights, with full autonomy achieved through the provision of supports, when required. The chapter nevertheless anticipated complexities shrouded by the simplicity of this framing. It rejected the strict social model of disability and endorsed a social model that recognises the residual impacts of impairment.

Chapter 3 introduced the *principle of the indivisibility of human rights*, settling on an understanding of indivisibility that acknowledged interdependencies between rights and recognised socio-economic as of equal importance as civil-political rights. It further demonstrated how the principle of indivisibility infuses the text of the CRPD more than any other human rights convention. It identified that the right to legal capacity in Article 12 was widely considered a civil-political right. As such, Article 12 is typically conceived of as being achieved through negative state 'action' – in particular, restraint from the imposition of guardianship. Such restraint ostensibly respects the autonomy of the individual and his or her personal realm. However, the chapter further argued that, in requiring the positive provision of socio-economic supports, Article 12 incorporates a mix of interdependent civil-political and socio-economic rights. In doing so, the article recognises and embodies the principle of indivisibility. This current chapter investigates how, in disrupting the traditional categorisation of rights, Article 12 also disrupts the traditional concept of autonomy, which has been privileged together with the privileging of civil-political rights.

Section 2 recaps but expands upon the concept of autonomy introduced previously to explain how autonomy is traditionally conceived of as Kantian rationality and Millian liberty, demanding restraint from state interference. It reiterates how autonomy as rationality and independence underpins liberal conceptions of personhood, thereby excluding adults with cognitive disability. It further describes the challenge that this equation of rationality-with-autonomy-with-personhood has posed for disability advocates and scholars who aim to recognise adults with cognitive disability as persons.

Sections 3 and 4 describe several variants of one broad strategy to address this challenge ('the first strategy'). This first strategy still operates within the liberal tradition in privileging the value of autonomy above all other values as the basis for personhood, and the associated right to legal capacity above all other rights, but reworks, or reinvents, the concept of autonomy to include adults with cognitive disability. Autonomy is refashioned as ostensibly universal by divorcing it from rationality, reconceptualising it as *inter*dependence, and framing it as a combination of negative and positive liberty. This strategy supports an interpretation of Article 12 that requires the abolition of decision-making by substitutes to ensure the recognition of universal autonomy and legal capacity. However, I conclude that these refashionings fail because of various internal contradictions in reasoning, all linked to the resistance to recognising the residual impacts of cognitive impairment.

Section 5 describes a second broad strategy to include adults with cognitive disability as persons, this time by divorcing autonomy from personhood and allowing space for alternative bases, including dignity ('the second strategy'). This second strategy endorses or allows for an interpretation of Article 12 that, without denying personhood, permits decision-making by substitutes as a last resort where a person is non-autonomous because of cognitive impairment or disability. According to this strategy, autonomy is no longer privileged above other values; however, because it is still *highly* valued, commentators rework the concept of autonomy to be more inclusive. I conclude that a concept of autonomy as *achievement* – which recognises the need for substantive autonomy competencies and also incorporates a demand for the provision of real options – best aligns with Article 12 autonomy.

Section 6 explains how a substantive concept of autonomy as the achievement of competencies and as demanding options can be rearticulated in human rights terms as recognising the indivisibility of human rights. Autonomy in the sense of achievement of competencies no longer underpins just civil-political rights and is no longer marked by those features typically associated with civil–political rights. That is, autonomy is not always realised through negative state action, is not cost-free, may be progressively realisable and may also require intervention in the personal sphere. Section 7 argues that the concept of autonomy underpinning Article 12 is one that recognises the indivisibility of human rights. Article 12 autonomy acknowledges that not just supported decision-making but decision-making by substitutes may be required to realise a broad range of civil, social, political, economic and cultural rights to respect and achieve autonomy.

The chapter uses the case study of Alex introduced in Chapter 1 and reproduced below, to illustrate the different theories of autonomy, how they affect decision-making and personhood and how they intersect with human rights.

ALEX

Alex is a 30-year-old man with an intellectual disability who refuses dental treatment because he is scared of, and doesn't trust, the dentist. Collateral information is that his fear is linked to post-traumatic stress disorder from childhood abuse. Counsellors have been unable to assuage his fears or convince him to have the treatment. They have warned him that he will lose all of his teeth if he does not have the treatment. His support workers and family all know that one of his real joys in life is to partake of gourmet food, and it is a significant social interest that he can share with others. If Alex loses all of his teeth, he will need to live chiefly on pureed food. It is not clear whether he understands this longer-term adverse impact (as opposed to the shorter-term impact of undergoing the treatment). It appears to Alex's supporters that he thinks that his teeth will get better on their own, and he will be able to eat steak again soon.

2 THE TRADITIONAL LIBERAL VIEW OF AUTONOMY: 'TO LIVE ONE'S LIFE ACCORDING TO REASONS AND MOTIVES THAT ARE TAKEN AS ONE'S OWN'[1]

Chapter 2 described how legal capacity has served as the 'gateway for autonomy'[2] and introduced the concept of autonomy in philosophy and law.[3] It referred, in particular, to the writings of Kant and Rawls, whose constructs of moral personhood and citizenship are based on the recognition of autonomy. If, as Kant proposes, autonomy is the foundation of personhood, then logically the consequence for individuals who are non-autonomous and who have decisions made for them by substitutes is that they are non-persons.[4]

Dworkin famously wrote: 'About the only features held constant from one author to another are that autonomy is a feature of persons and that it is a

[1] John Christman, 'Autonomy in Moral and Political Philosophy' in Edward N Zalta (ed), *The Stanford Encyclopedia of Philosophy* (Fall 2020 Edition), https://plato.stanford.edu/archives/fall2020/entries/autonomy-moral/

[2] Mary Donnelly, *Healthcare Decision-Making and the Law: Autonomy, Capacity and the Limits of Liberalism* (Cambridge University Press, 2010) 90.

[3] See Chapter 2, Section 2.1 'Charity Model', Section 3.2.3 'Functional Test', and Section 3.3.2 'Substituted Judgement and Will and Preferences'.

[4] See Chapter 2, Section 2.1.

desirable quality to have.'[5] Feinberg expressed his doubts that autonomy has a single, coherent meaning.[6] An extensive description and analysis of the various conceptions and critiques of autonomy are beyond the scope of this book, which instead focuses on autonomy as understood in the Article 12 literature. For this purpose, Christman's definition provides a useful starting point:

> Individual autonomy is an idea that is generally understood to refer to the capacity to be one's own person, to live one's life according to reasons and motives that are taken as one's own and not the product of manipulative or distorting external forces.[7]

Christman's definition reveals that autonomy is associated with three dominant and overlapping features: rationality or the ability to reason, living one's life independently and being free from interference by others. I elaborate on these three features below before considering their application in Article 12 decision-making.

2.1 *Autonomy as Rationality*

For Kant, rational agency and autonomy are pre-conditions for an individual to be respected by others and recognised as having moral personhood.[8] Rawls draws on the legacy of Kant in his theory of justice as fairness by prioritising the ability for rational thinking by individuals in developing a just and fair society.[9] He theorises that such a society is based on a social contract negotiated by persons who make decisions freely and voluntarily using their ability to exercise rational judgement to develop and pursue their own conception of the good life.[10] Rawls's idea of the social contract is based on the premise that:

> each person beyond a certain age and possessed of the requisite intellectual capacity develops a sense of justice … We acquire a skill in judging things to be just and unjust and in supporting these judgments by reasons.[11]

For the exercise of rational judgement and the ability to bargain, a 'requisite' level of 'intellectual capacity' is required.[12] Because adults with cognitive disability may

[5] Gerald Dworkin, *The Theory and Practice of Autonomy* (Cambridge University Press, 1998) 6.
[6] John Christman, 'Constructing the Inner Citadel: Recent Work on the Concept of Autonomy' (1988) 99(1) *Ethics* 109, 109.
[7] Christman, 'Autonomy in Moral and Political Philosophy' (n 1) [1].
[8] David T Wasserman et al, 'Cognitive Disability and Moral Status' in Edward N Zalta (ed), *The Stanford Encyclopedia of Philosophy* (Fall 2017 Edition), <https://plato.stanford.edu/archives/fall2017/entries/cognitive-disability/>; Christman, 'Autonomy in Moral and Political Phjilosphy' (n 1) [1.1].
[9] John Rawls, *A Theory of Justice* (Harvard University Press, 1971) 3, 11–12.
[10] Ibid 16.
[11] Ibid 46.
[12] See also earlier account of Rawls in Chapter 2, Section 2.1.

lack the ability to reason, they are considered non-autonomous and non-persons.[13] Functional tests of capacity typically test what we would consider an adult's rational capacities. They consider whether an adult can understand information relevant to a decision; retain, use and weigh that information to make the decision; and communicate that decision in some way through any means.[14]

2.2 *Autonomy as Independence*

The traditional liberal accounts of autonomy are of the independent, rational individual or citizen as an isolate who can negotiate and bargain his or her position in the social contract. In acting according to his or her own values, the autonomous individual acts independently, unbowed by the influence of others. In this sense, classic liberal autonomy constitutes the capacity for '*procedural independence* or *independence of mind*' in requiring certain processes of critical reflection without dictating external, substantive values.[15] Autonomy in a procedural sense demands that a person's autonomous choices or actions be free from 'coercion or manipulation by others',[16] so an autonomous individual has strong boundaries between self and others. Mill's conception of liberty has greatly influenced contemporary concepts of autonomy, privileging the value of the individual in society and the individual's freedom to make personal life choices without interference from others. While Mill also proposed limited exceptions to the privileging of liberty in cases where the exercise of individual liberty could result in harm,[17] proceduralist autonomy is considered by many to have intrinsic value quite apart from any instrumental benefits it may bring and even if it results in harm to the self or others.[18]

2.3 *Autonomy as Non-interference in the Private Sphere*

Linked with the concept of individual autonomy in the liberal philosophy of both Mill and Rawls is a clear distinction between the public and private spheres of life.

[13] People who are non-autonomous include 'not only the permanently unconscious, but also those, such as the profoundly and permanently demented, who are not persons and who have only radically truncated interests'. Buchanan, Allen E and Dan W Brock, *Deciding for Others: The Ethics of Surrogate Decision Making* (Cambridge University Press, 1989) 12.

[14] Ben White, Lindy Willmott and Shih-Ning Then, 'Chapter 7: Adults Who Lack Capacity: Substitute Decision Making' in Ben White, Fiona McDonald and Lindy Willmott (eds), *Health Law in Australia* (Lawbook Co, 3rd ed, 2018) 207, 229.

[15] Catriona Mackenzie and Natalie Stoljar, 'Introduction: Autonomy Reconfigured' in Catriona Mackenzie and Natalie Stoljar (eds), *Relational Autonomy: Feminist Perspectives on Autonomy, Agency and the Social Self* (Oxford University Press, 2000) 3, 40.

[16] Ibid.

[17] John Stuart Mill, *On Liberty* (Routledge, 1991) 30.

[18] Viv Ashley, *Philosophical Models of Personal Autonomy (version 5)* (Essex Autonomy Project, 2013) 11.

For Mill, state interference compromises individual liberty.[19] For Rawls, the public, political sphere is situated outside of the family – the social contract is negotiated between adult men as citizens and subjects,[20] leaving the private, family sphere as an apolitical space.[21] In traditional liberal philosophy, the distinction between the individual and the State is critical. As described in Chapter 3, civil–political rights to non-interference in the personal, private realm are thereby privileged while socio-economic rights that intervene in the personal realm and impose external values are devalued as not 'true' rights.[22] Given the history of state institutionalisation of people with disability and associated abuses, it is not surprising that disability advocates are suspicious of state power.[23] In this context, the privileging of the right of people with disability to be left alone, and to be allowed to have personal, private lives without state involvement, is an important one. Guardianship and mental health legislation typically require implementation in the 'least restrictive' way in deference to the concept of autonomy as lack of interference.[24]

2.4 *The Case of Alex*

In the case of Alex, let us assume that he has been assessed according to a functional test as not having the capacity to make the decision relating to the dental treatment. He does not understand the longer-term benefits of having the treatment and cannot weigh those benefits against the shorter-term pain. A substitute decision-maker could be appointed to make a decision in Alex's best interests. The substitute may well decide that the dental work should proceed, having considered both Alex's expressed wishes and the longer-term benefits of treatment.

If, on the other hand, decision-making by substitutes were no longer an option, then Alex would instead be supported to make a decision that would be attributed to him as his own. The possible different outcomes of a process of supported decision-making are investigated further in the sections below. They show that the challenge

[19] Mill (n 17) 30.

[20] Dodds writes of Rawls that: 'The principles of justice were only applied to activities in the public realm': Susan Dodds, 'Depending on Care: Recognition of Vulnerability and the Social Contribution of Care Provision' (2017) 21(9) *Bioethics* 500, 501.

[21] See Camillia Kong, 'The Convention on the Rights of Persons with Disabilities and Article 12: Prospective Feminist Lessons against the "Will and Preferences" Paradigm' (2015) 4(4) *Laws* 709, 608–9.

[22] See Chapter 3, Section 3.3.

[23] Rosemary Kayess and Philip French, 'Out of Darkness into Light? Introducing the Convention on the Rights of Persons with Disabilities' (2008) 8(1) *Human Rights Law Review* 1, 12.

[24] Barbara Carter, 'The Case for Dignity as the Governing Principle in Adult Guardianship' (2010) 19(1) *Res Publica* 1, 3; for explanation of the 'least restrictive way' in mental health and guardianship legislation, see: Julia Duffy, Sam Boyle and Katrine Del Villar, 'What Does "Least Restrictive" or "Less Restrictive" Mean in Mental Health Law? Contradictions and Confusion in the Case of Queensland, Australia' (forthcoming 2023, symposium issue) *American Journal of Law and Medicine*.

for supported decision-making in this and other cases is that, under a traditional liberal view, this support could also be conceptualised as compromising Alex's autonomy. It would compromise his autonomy by limiting his independence, interfering in his personal/private realm, and potentially undermining his own value system. For this reason, advocates of universal legal capacity have sought to refashion autonomy by framing it as underpinning all supported decision-making and therefore inclusive of all adults with cognitive disability.

Sections 3 and 4 describe how the literature advocating universal legal capacity refashions autonomy to include adults with cognitive disability. It is refashioned by divorcing it from rationality; reconceptualising independence as interdependence; and reframing Millian liberty (freedom from interference) to a mix of 'positive' and 'negative' liberty. Following these descriptions, I return to the case of Alex to argue why and how none of these refashionings functions convincingly to reconceptualise autonomy as an inclusive basis for personhood.

3 AUTONOMY AS PERSONHOOD: BUT REFASHIONED BY DIVORCING AUTONOMY FROM RATIONALITY

Kantian autonomy underpins a concept of legal capacity as the right to make choices in accordance with one's own values, but, according to Kant, such choices must be rational. So, in refashioning autonomy to be inclusive of all adults with cognitive disability, many commentators have sought to de-emphasise or subordinate the role of rationality in decision-making.

3.1 *A Role for Emotions in Decision-Making and Autonomy*

General Comment No 1 criticises functional tests of capacity because they rely on testing mental capacity, a concept that 'is highly controversial in and of itself. Mental capacity is not, as is commonly presented, an objective, scientific and naturally occurring phenomenon.'[25] In this statement, the UN Disability Committee suggests that decision-making does not always rely on mental capacity as rationality. In the literature more widely, there is some controversy as to what extent decision-making processes are wholly cognitive as opposed to emotional or more broadly psychological, and therefore subject to internal influences that affect otherwise purely cognitive or rational functioning.[26]

[25] *United Nations Committee on the Rights of Persons with Disabilities 'General Comment No. 1 (2014) Article 12: Equal Recognition Before the Law CRPD/C/GC/1' (2014)* ('*General Comment No 1*') [13]–[15].

[26] Ashley (n 18) 11–12; Marina Oshana, 'Personal Autonomy and Society' (1998) 29(1) *Journal of Social Philosophy* 81, 81; George Szmukler, Rowena Daw and Felicity Callard, 'Mental Health Law and the UN Convention on the Rights of Persons with Disabilities' (2014) 37(3)

Some theorists argue strongly that emotional and affective factors play a significant role in decision-making, even for those who have the capacity for high-level reasoning.[27] They assert that the boundary between reason and emotion is more porous than Kantian philosophy or science allows.[28] Such arguments have been raised or relied on by those seeking to include adults with cognitive disability within the conceptions of autonomy and personhood that underpin Article 12.[29] Even though such adults may lack rational decision-making abilities, they nevertheless can exercise choice and make decisions in accordance with their own emotionally derived and constituted values, with the added implication that all of us are influenced by emotional factors in our decision-making.

In the case of Alex, if we challenge the dominance of rationality as a basis for personhood, we could ascribe capacity and personhood to Alex. There is indeed a strong emotional 'logic' to his resistance to dental treatment, given his past traumatic experiences. In this case, Alex's legal capacity would be recognised, the treatment would not proceed, and Alex would lose his teeth.

3.2 *Rationality Not Needed for Exercising 'Mere Choice' or 'Preference'*

Kant's concept of the will has been described as 'distinct from the ability to choose'. This is because the exercise of our Kantian will is constrained by our obligation to act according to the moral law and to make decisions based on reason.[30] Extrapolating

International Journal of Law and Psychiatry 245, 248; in David Bilchitz, 'Dignity, Fundamental Rights and Legal Capacity: Moving beyond the Paradigm Set by the General Comment on Article 12 of the Convention on the Rights of Persons with Disabilities' (2016) 32(3) *South African Journal on Human Rights* 410, 425; Christman writes that 'an answer to the question of whether (and to what extent) an agent must be rational to be autonomous has not been settled on in the literature': Christman, 'Constructing the Inner Citadel' (n 6) 116. The amount of literature in this area is vast, but another influential and much cited text is Daniel Kahneman, *Thinking, Fast and Slow* (Farrar, Strauss and Giroux, 2011).

[27] Donnelly (n 2) 99–100; 'Autonomous decision making is therefore built upon the interaction between cognition (giving practical reason content) and emotion (giving practical reason vision)': in Jonathan Herring and Jesse Wall, 'Autonomy, Capacity and Vulnerable Adults: Filling the Gaps in the Mental Capacity Act' (2015) 35(4) *Legal Studies* 698, 705.

[28] Mackenzie and Stoljar (n 15) 38.

[29] Gerard Quinn and Abigail Rekas-Rosalbo, 'Civil Death: Rethinking the Foundations of Legal Personhood for Persons with a Disability' (2016) 56 *Irish Jurist* 286, 322; Arstein-Kerslake writes of autonomy and personhood in the CRPD: 'It goes beyond traditional liberal notions, which restrict personhood to individual actors operating on the basis of "rationally" conscripted conceptions of their own "good"' in Anna Arstein-Kerslake, 'An Empowering Dependency: Exploring Support for the Exercise of Legal Capacity' (2016) 18 *Scandinavian Journal of Disability Research* 77, 78; Gerard Quinn, 'From Civil Death to Civil Life: The Decay and Re-Birth of Legal Capacity Law around the World' (Conference Paper, Australian Guardianship and Administration Council National Conference: Reflecting Will and Preference in Decision-Making, 17–18 October 2016) 11, <www.agac.org.au/images/stories/2016nsw/quinn-decay-rebirth-legal-cap-law.pdf>.

[30] Jerome Borges Schneewind, 'Kant on the Will' in Thomas Pink and Martin William Francis Stone (eds), *The Will and Human Action: From Antiquity to the Present Day* (Routledge, 2004)

from Kant, Frankfurt argues that the ability to have a will is what constitutes us as humans and is evidenced by our ability to reflect on our 'first-order desires' to develop and act on our 'second-order volitions'.[31] For example, our first-order desire may be to have a piece of chocolate cake, but on reflection, our second-order volition may be to refuse the cake for health reasons. Frankfurt's theory of autonomy is one of several referred to as 'hierarchical accounts', which link autonomy and the notion of the will with the capacity to reflect and reason.[32]

Szmukler applies a hierarchical account of autonomy to explain the meanings of 'will' and 'preference' as they relate to Article 12 decision-making. He argues that a person's will relates to a decision that is formed through higher-order executive functioning, and thus reflects 'a person's deep beliefs, values and commitments', whereas a preference is a more fleeting desire, and not therefore reflective of a person's autonomous will. Szmukler argues that in Article 12 terms, autonomy, legal capacity and the ability to make decisions require the ability to reflect rationally on one's preferences; and when there is a conflict or divergence between an adult's will and their preferences, the former should prevail.[33]

The idea of autonomy as the right to have choices and make decisions has dominated medical ethics and can be seen in its emphasis on the right to consent to or refuse treatment.[34] This notion of autonomy has also been shaped by Mill's idea of individual liberty as the right to make choices and take actions without interference from others, to pursue a life plan that aligns with our own 'character'.[35] Emphasis on autonomy as the right to make choices is foregrounded in the CRPD's Preamble, which recognises 'the importance for persons with disabilities of their individual autonomy and independence, including the freedom to make their own choices',[36] and the General

154, quoted George Szmukler, '"Capacity", "Best Interests", "Will and Preferences" and the UN Convention on the Rights of Persons with Disabilities' (2019) 18(1) *World Psychiatry: Official Journal of the World Psychiatric Association (WPA)* 34, 38.

[31] Harry Frankfurt, 'Freedom of the Will and the Concept of a Person' (1971) 68(1) *Journal of Philosophy* 5, 6–18.

[32] For example, Dworkin describes how autonomy is constituted by being able to act upon our second-order desires, which are derived from our ability to critically reflect upon our 'first-order preferences, desires, wishes, and so forth and the capacity to accept or attempt to change these in the light of higher-order preferences and values'. Dworkin (n 5) 20. See, however, some criticism of hierarchical accounts in Tom L Beauchamp and James F Childress, *Principles of Biomedical Ethics* (Oxford University Press, 6th ed, 2009) 101. Also criticised in Diana Meyers, *Self, Society and Personal Choice* (Columbia University Press, 1989) 33–4.

[33] George Szmukler, 'The UN Convention on the Rights of Persons with Disabilities: "Rights, Will and Preferences" in Relation to Mental Health Disabilities' (2017) 54 *International Journal of Law and Psychiatry* 90, 96. See also in relation to Article 12, a discussion on the difference between immediate preference and 'higher-order loading of more settled values', in Terry Carney et al, 'Realising "Will, Preferences and Rights": Reconciling Differences on Best Practice Support for Decision-Making?' (2019) *Griffith Law Review* 357, 360.

[34] Beauchamp and Childress (n 32) 101.

[35] Mill (n 17) 33.

[36] *CRPD* Preamble para 14.

Principles in Article 3, which affirm 'Respect for … individual autonomy including the freedom to make one's own choices, and independence of persons.'[37]

However, some interpretations of Article 12 as the right to choose reflect a less demanding concept of choice than that envisaged by Kant, Mill, Frankfurt or Szmukler. In this way, autonomy or legal capacity has become the right to have any desires or choices recognised at law and not just decisions made according to a process of rational deliberation reflecting an adult's will.[38] Hall is critical of such a thin concept of autonomy. She draws a distinction between 'mere choices' made by an adult with advanced dementia (which in her view may be legitimately overridden by the adult's guardian) and considered decisions that should be legally recognised as the adult's own.[39]

In the case of Alex, if he were to make a decision according to his 'mere choice', then his immediate desire and preference would be to refuse treatment. Such a decision may, however, conflict with his underlying values or will, relating to the joy he finds in dining with his friends.

3.3 *Rationality Not Needed for a Person to Have an 'Authentic' Self*

The hierarchical theories of autonomy described above suggest we have an 'authentic' self.[40] In this sense, they are premised on the idea that a person's actions and decisions have a diachronic coherence, sometimes referred to in the Article 12 literature as an adult's coherent 'life narrative'.[41] The requirement that, in cases where an adult's will and preferences cannot be determined, a decision should be made according to the 'best interpretation' of their will and preferences appears to be premised on the assumption of authenticity and narrative coherence. Bach and Kerzner, for example, write:

[37] See also: Article 19 (Living independently and being included in the community), which recognises the right of persons to live in the community 'with choices equal to others'; Article 20 (Personal mobility), which requires mobility to be facilitated 'in the manner and at the time of their choice'; Article 21 (Freedom of expression and opinion, and accessible information), which requires that people have 'all forms of communication of their choice'; and Article 29 (Participation in political and public life), which provides that a person may have an assistant 'of their own choice' to facilitate voting in elections.

[38] World Network of Users and Survivors of Psychiatry, 'Implementation Manual for the United Nations Convention on the Rights of Persons with Disabilities' (February 2008) 16, 20.

[39] Margaret Hall, 'Rethinking Adult Guardianship: Mental Capacity and Vulnerability in the Context of Dementia in Old Age' (PhD Thesis, University of British Columbia, 2018) 13; see also: Katrine Del Villar, 'Should Supported Decision-Making Replace Substituted Decision-Making? The Convention on the Rights of Persons with Disabilities and Coercive Treatment under Queensland's Mental Health Act 2000' (2015) 4(2) *Laws* 173, 186.

[40] Christman, 'Autonomy in Moral and Political Philosophy' (n 19) [1]; Dworkin (n 5) 15.

[41] Lucy Series, 'Relationships, Autonomy and Legal Capacity: Mental Capacity and Support Paradigms' (2015) 40 *International Journal of Law and Psychiatry* 80, 86; Kristin Booth Glen, 'Introducing a New Human Right: Learning from Others, Bringing Legal Capacity Home' (2018) 49(3) *Columbia Human Rights Law Review* 1, 41.

> If to my actions can be ascribed intentions of my will which themselves can be woven into a coherent narrative, either by myself or others, even if the intentions or wilful behaviour represent some discontinuity with the past, then this should be sufficient ground for exercising my legal capacity.[42]

Glen similarly suggests that a 'supporter', making a decision under an advance directive, could 'make the choice consistent with the person's "general life narrative"',[43] and Quinn writes that Article 12 represents a shift 'to ensure that the authentic voice of the person is allowed to come through loud and clear'.[44]

In the case of Alex, if we were to recognise his capacity under Article 12 and then apply a hierarchical account, we could say that his preference not to have the treatment is a mere choice or desire that he hasn't reflected on, and so it doesn't represent his autonomous will. His autonomous will is expressed in the longer-term desire to eat and socialise with his friends. This social activity is part of his authentic self, according to his coherent life narrative. Under this account, the supporter would interpret his autonomous will to be to have the treatment, even though this would contradict his expressed preferences.

Alternatively, it could be argued that Alex's survival of sexual abuse as a child is part of his life narrative; that he doesn't want to just dismiss this as irrelevant to the way he conducts himself, but that it is part of his authentic self. On this account, given that the anticipation of dental treatment brings back memories of this sexual assault, a supporter would interpret his will as being to refuse the treatment, such refusal also being in accordance with his expressed preference.

If a 'best interpretation' of an adult's will and preferences is one that accords most clearly with the adult's values, according to his or her authentic self and consistent with his or her life narrative, then different interpretations will lead to alternative and potentially conflicting decisions.

3.4 *Why Autonomy without Rationality Does Not Work as Universal Personhood*

Commentators who advocate for the abolition of decision-making by substitutes argue that emotions play a significant role in decision-making and that an adult's 'mere choice' or preferences should be respected as their decision and as an exercise of legal capacity under Article 12. Consistently with this, a 'best interpretation' of an adult's will and preferences is one that reflects their authentic self or their coherent life narrative, whether such self or life are constituted by thinking rationally or by feeling emotionally.

[42] Michael Bach and Lana Kerzner, *A New Paradigm for Protecting Autonomy and the Right to Legal Capacity: Advancing Substantive Equality for Persons with Disabilities through Law, Policy and Practice* (Law Commission of Ontario, October 2010) 63–6.

[43] Glen 'Introducing a New Human Right' (n 41) 41.

[44] Quinn (n 29) 4, 24.

However, the example of Alex shows the challenges of such an approach, when a supporter is presumed to know an adult's authentic self even better than the adult themselves, and in the face of unforeseen choices. Application of the concept of 'narrative coherence' to ascribe capacity to an adult under Article 12 has been criticised for falsely attributing an unconflicted unity to the claimant of rights,[45] failing to consider external factors that might influence the adult to develop a harmful narrative,[46] and overlooking that a supporter has 'considerable power to alter a person's "narrative" through their interpretation'.[47] It also assumes that an individual's will or decision is there to be 'discovered' by the supporter and not created or derived through the interactive process of supported decision-making itself.[48]

This notion of an authentic self is a strong theme in the Article 12 commentary, but it poses two critical challenges. The first is that if autonomy is divorced from rationality or self-reflection, relying on an adult's first-order desires or preferences can lead to different outcomes or 'decisions'. The second challenge relates to determining when an adult's desires are truly their own – that is, not unduly influenced by another, including the supporter.[49] Excising rationality from autonomy is one strategy for refashioning autonomy (and therefore legal capacity) as universal. However, I argue that this strategy effaces the difference that the residual impacts of cognitive impairment may have on decision-making ability. It also stretches the concept of autonomy beyond its meaningful limits.

4 AUTONOMY AS PERSONHOOD: BUT REFASHIONED AS '*INTER*DEPENDENCE' OR RELATIONAL AUTONOMY, RELATIONSHIPS OF TRUST OR AS POSITIVE AND NEGATIVE LIBERTY

As described above, traditional accounts of autonomy emphasise independence 'as self-sufficiency, individual self-determination, and freedom from the constraint and influence of others'.[50] However, Article 12(3) explicitly provides for support in exercising legal capacity. It, therefore, acknowledges that people with disability are not always independent, self-sufficient or unaffected by others in their decision-making

[45] Paul Skowron, 'Giving Substance to "the Best Interpretation of Will and Preferences"' (2019) 62 *International Journal of Law and Psychiatry* 125, 133.

[46] Camillia Kong, *Mental Capacity in Relationship: Decision-Making, Dialogue and Autonomy* (Cambridge University Press, 2017) 65.

[47] Series (n 41) 87.

[48] Carney et al (n 33) 360–1.

[49] Lucy Series and Anna Nilsson, 'Article 12 CRPD: Equal Recognition before the Law' in Ilias Bantekas, Michael Ashley Stein and Anastasiou Dimitris (eds), *The UN Convention on the Rights of Persons with Disabilities: A Commentary* (Oxford University Press, 2018) 339, 370. Note: 'All people risk being subject to "undue influence", yet this may be exacerbated for those who rely on the support of others to make decisions'. In *General Comment No 1* [22].

[50] Laura Davy, 'Philosophical Inclusive Design: Intellectual Disability and the Limits of Individual Autonomy in Moral and Political Theory' (2015) 30(1) *Hypatia* 132, 138.

processes and that support can enhance their autonomy and decision-making ability. To include adults with cognitive disability in conceptions of autonomy and personhood, some commentators have refashioned autonomy so that it is constituted by shared personhood or *inter*dependence; relational autonomy; cooperative relationships of trust, or as both positive and negative liberty. Sections 4.1–4.5 below provide a summary of some of these theories, while Section 4.6 argues why and how they ultimately fail to justify recognition of universal legal capacity or serve as a foundation for acknowledging inclusive personhood. Section 4.7 returns to the case study of Alex to illustrate these limitations.

4.1 *Autonomy as Shared Personhood or* Inter*dependence*

Adults with disability have been culturally marked as dependent, such dependency being a significant factor in society's protective attitude towards them and the associated denial of their autonomy and legal capacity. By contrast, Bach and Kerzner describe the autonomy envisaged by Article 12 as exhibiting 'interdependence' or as incorporating the notion of interdependence.[51] In doing so, they reconfigure liberal autonomy by ostensibly challenging the notion of the bounded and independent individual as the ideal of Kantian personhood.

Quinn and Arstein-Kerslake describe how the CRPD shifts the balance from protection to autonomy. They criticise the various capacity tests as failing to acknowledge that all of us 'rely on a wide variety of supports, social cues and the benefits of social capital to give context, meaning and direction to [our] choices' and that decision-making with others is the norm for all of us.[52] Quinn describes how the CRPD, with Article 12 at its epicentre, restores moral authority and personhood to adults with cognitive disability by acknowledging that 'our personhood' is 'primarily shared'[53] and that 'our autonomy is fundamentally relational'.[54] For these writers, Article 12 articulates a concept of 'shared'[55] personhood through its promotion of supported decision-making.

These writers seek to destigmatise dependence, reconceive it as 'interdependence' or 'shared personhood' and value it as essential to autonomy.[56] Dependence

[51] 'Rather autonomy is a social and inter-dependent accomplishment': Bach and Kerzner (n 42) 95.

[52] Gerard Quinn and Anna Arstein-Kerslake, 'Restoring the "Human" in "Human Rights": Personhood and Doctrinal Innovation in the UN Disability Convention' in Costas Douzinas and CA Gearty (eds), *Cambridge Companion to Human Rights Law* (Cambridge University Press, 2012) 36, 46.

[53] Quinn (n 29) 12.

[54] Ibid 10.

[55] Quinn and Arstein-Kerslake (n 52) 40.

[56] Bach and Kerzner (n 42) 96; Quinn (n 29) 10–12; Quinn and Rekas-Rosalbo (n 29) 17; Andrea Nicki, on the other hand, points out the limitations of a sense of self based on interdependence when the 'connectedness' in an abusive relationship leads to a diminishing sense of self: 'For people … from severely abusive families, connectedness is a great source of despair and

has historically been stigmatised both by the wider community and by disability advocates themselves, so autonomy as interdependence endeavours to normalise, render mainstream and universalise the 'dependencies' of disability.[57] Autonomy as interdependence is used to include all adults with cognitive disability as persons.

4.2 *Relational Autonomy*

Bach and Kerzner, Arstein-Kerslake, Quinn and others also invoke or refer to the notion of 'relational autonomy' to describe the autonomy envisaged by Article 12. Bach and Kerzner refer to Mackenzie and Stoljar, who describe 'relational autonomy' as 'an umbrella term, designating a range of related perspectives'.[58] Mackenzie and Stoljar describe the various theories of relational autonomy as diverse but as nevertheless 'premised on a shared conviction, the conviction that persons are socially embedded and that agents' identities are formed within the context of social relationships and shaped by a complex of intersecting social determinants, such as race, class, gender and ethnicity'.[59] Under some accounts of relational autonomy, an individual is said to be almost equivalent to the sum of their social relationships, so that 'the *identity of the self* is constituted by various social attachments'.[60]

Mackenzie and Stoljar go on to explain that some forms of feminism originally dismissed autonomy as inherently masculinist and, therefore, not a concept or condition to be given high value. Compared with men, most women lived lives of dependence (at least financial dependence), so the original feminist analysis proposed that values associated with dependency, such as trust, loyalty, friendship and caring, should be prioritised over independence and autonomy. The development of the concept of relational autonomy endorses autonomy as a normative value but rejects the 'hyperbolic abstraction' of autonomy as independent self-determination in favour of 'the possibilities for anti-individualistic conceptions'.[61]

Arstein-Kerslake explains relational autonomy as challenging traditional notions of atomistic selfhood and decision-making, and as therefore describing or allowing for a model of decision-making with support.[62] For Quinn, the concept

self-hatred': Andrea Nicki, 'The Abused Mind: Feminist Theory, Psychiatric Disability and Trauma' (2001) 16(4) *Hypatia* 80, 98.

[57] Barbara Arneil, 'Disability in Political Theory versus International Practice: Redefining Equality and Freedom' in Barbara Arneil and Nancy J Hirschmann (eds), *Disability and Political Theory* (Cambridge University Press, 2016) 10, 40–2.

[58] Bach and Kerzner (n 42) 39, quoting Mackenzie and Stoljar (n 15) 4.

[59] Mackenzie and Stoljar (n 15) 4.

[60] Ibid 61.

[61] Ibid 6–8.

[62] Anna Arstein-Kerslake, *Restoring Voice to People with Cognitive Disabilities: Realizing the Right to Equal Recognition before the Law* (Cambridge University Press, 2017) 63.

of relational autonomy shines a light on the interdependent nature of personhood more widely, an interdependence that is normally hidden and denied by the myth of 'the atomised rational man'.[63] Like Arstein-Kerslake, Quinn expressly applies this thinking to the paradigm of supported decision-making required by Article 12(3) CRPD, arguing that supports are not required as an exception to the norm and can therefore be provided without undermining or negating the personhood of adults with cognitive disability. For Quinn, supported decision-making should be embraced as describing and reflecting the practices of all of us.[64] Under these accounts, autonomy is reconceptualised as relational autonomy. An adult is autonomous to the extent that he or she is embedded in a network of social and personal relationships. Under this account, we are all autonomous (i.e. 'relationally' autonomous).

4.3 *Autonomy Characterised by Trust, Cooperation and the 'Cognitive Prosthesis'*

The reconceptualisation of liberal autonomy as a condition of interdependence has echoes in philosophical theories that attribute personhood through the notion of trusteeship.[65] Silvers and Francis develop a trusteeship model for adults with cognitive disability, which provides that an adult who is dependent on another (e.g. on a supporter) for decision-making should nonetheless be viewed as autonomous and attributed full moral personhood.

4.3.1 Trust and Cooperation

Francis begins by acknowledging that adults with cognitive disability do not necessarily have autonomy in the traditional Rawlsian sense but proposes a more complex conception of autonomy that includes them as persons and citizens. She lists some of the indicia typically ascribed to autonomy as follows: the capacity to value; the capacity to understand and reason; the ability to be an agent; the ability to formulate plans; the ability to act according to principles (not desires); the ability to not be subject to coercion; and the ability to act rationally.[66] Focusing on adults

[63] Quinn (n 29) 9; see also Cahill, who prefers the concept of relational autonomy to more traditional liberal accounts of autonomy, because in her view, it more readily recognises non-rational elements of decision-making, as exhibited by adults with dementia: Suzanne Cahill, *Dementia and Human Rights* (Policy Press at the University of Bristol, 2018) 14.

[64] Quinn (n 29) 10.

[65] Wasserman et al (n 8) describes theories of 'Proxy, derivative, and presumptive moral status', describing Thomas Scanlon as affirming full moral status to all human beings, including adults with cognitive disabilities who can acquire the capacity for 'judgement-sensitive' attitudes or moral personhood' 'vicariously, through trustees'; Leslie Pickering Francis also refers to Scanlon in: Leslie P Francis, 'Understanding Autonomy in Light of Intellectual Disability' in Kimberley Brownlee and Adam Cureton (eds), *Disability and Disadvantage* (Oxford University Press, 2009) 200, 210.

[66] Francis (n 65) 201.

with intellectual disability, she proposes that they may not possess all of these attributes but still may be autonomous.[67]

Silvers and Francis also propose a refashioning of Rawls's social contract by questioning his view of society as being formed by atomistic individuals, each involved in negotiations with one another. They propose instead that the social contract can more usefully be conceived of as a network of relationships developed through a process of trusting rather than bargaining.[68] Trust, according to these authors, is a quality that everyone can possess, independent of cognitive ability or rational capacity, thereby including those with cognitive disability as moral persons. While we are all individuals who, in turn, represent ourselves to others, we are also 'trustees' for others who represent others to society in a certain way, so that 'autonomy is so often a collaborative affair'.[69] Hartley likens this proposal to her own conception of the social contract as comprising relationships of 'cooperation for mutual advantage'.[70] She endorses a 'relationship model' of cooperation so that when adults cannot contribute in ways that can be readily quantified (e.g. in economic or financial terms), they nevertheless 'can contribute to society too, through the capacity for engagement'.[71] Because people with cognitive disability make a positive cooperative contribution to social relationships, they are within the social contract and personhood is attributed to them.[72] Autonomy is reconceived as a network of mutually trusting relationships that mark our personhood and in which we all participate.

4.3.2 The Supporter as Cognitive Prosthesis

Arstein-Kerslake describes how Francis and Silvers alternatively propose a 'prosthesis model' of personhood. According to this model, a support person for an adult with cognitive disability is a 'cognitive prosthesis'[73] for the adult, so as to allow that adult to convey their own conception of 'the good' and thereby exercise their autonomy. To

[67] Ibid 207.

[68] Anita Silvers and Leslie Pickering Francis, 'Justice through Trust: Disability and the "Outlier Problem" in Social Contract Theory' (2005) 116(1) *Ethics* 40, 45.

[69] Francis (n 65) 215. Arstein-Kerslake endorses the trusteeship model of Silvers and Francis, as being consistent with the concept of autonomy that underlies the CPRD and Article 12: Arstein-Kerslake, *Restoring Voice to People with Cognitive Disabilities* (n 62) 43–4; Arstein-Kerslake, 'An Empowering Dependency' (n 29) 78.

[70] Christie Hartley, 'An Inclusive Contractualism: Obligations to the Mentally Disabled' in Kimberley Brownlee and Adam Cureton (eds), *Disability and Disadvantage* (Oxford University Press, 2009) 138, 143.

[71] Ibid 148.

[72] Ibid 147ff; see also discussion of Hartley in Stacy Clifford Simplican, 'Disavowals of Disability in Rawls' Theory of Justice and His Critics' in Barbara Arneil and Nancy J Hirschmann (eds), *Disability and Political Theory* (Cambridge University Press, 2016) 79, 91–2.

[73] Arstein-Kerslake, 'An Empowering Dependency' (n 29) 78; Leslie P Francis and Anita Silvers, 'Thinking about the Good: Reconfiguring Liberal Metaphysics (or not) for People with Cognitive Disabilties' in Eva Feder Kittay and Licia Carlson (eds), *Cognitive Disability and the Challenge to Moral Philosophy* (2010) 237, 249.

ensure the adult's liberty and autonomy, trustees as 'prostheses' must represent the adult's views and preferences and not project their own onto the adult.[74] For Arstein-Kerslake, this is what marks the trusteeship/prosthesis model as underpinning and justifying a supported and not a substitute decision-making model.[75] According to the theory, the decision is the adult's own, despite the involvement of the prosthesis. If an adult is supported by a prosthesis, then he or she can have autonomy and be recognised as a person like any other, with any decision made being attributed to the adult and not to the prosthesis/supporter. Francis and Silvers describe how this process of assistive thinking, as an interaction between the 'cognitively disabled individual' and the trustee or prosthesis, enables the individual to be included in Rawls's social contract, 'revealing' their personhood.[76]

4.4 *Supports as Enabling Positive Liberty/Autonomy*

As described above, Bach and Kerzner conceptualise Article 12 autonomy as interpersonal and relational, but they also describe it as incorporating a concept of 'positive liberty'. They explain that autonomy is usually conceived of as 'negative liberty' (i.e. as the absence of restraints[77]) but suggest that Article 12 invokes a 'positive liberty' view of autonomy because it is created by the positive provision of supports.[78] They acknowledge a tension between the two concepts of positive and negative liberty but argue that a careful balancing of the two is required so that adults with cognitive disability are not left as objects of abuse and neglect. Positive liberty means that: 'the state has a positive obligation to maximise the exercise and enjoyment of autonomy by providing individuals with the goods and services they require for this purpose, and for developing their own decision-making capabilities to exercise their autonomy'.[79] A contradiction begins to emerge as autonomy is traditionally conceived of as 'negative liberty', thus requiring non-interference by the State, and yet positive actions by the State are required to ensure autonomy.

[74] Arstein-Kerslake, 'An Empowering Dependency' (n 29) 78; see also: Leslie P Francis, Barbara E Bierer and Michael Ashley Stein, 'An Externalist, Process-Based Approach to Supported Decision-Making' (2022) 22(10) *The American Journal of Bioethics* 55, 56.

[75] Ibid.

[76] Francis and Silvers (n 73) 256; see also: Leslie Francis, 'Supported Decision-Making: The CRPD, Non-Discrimination, and Strategies for Recognizing Persons' Choices about Their Good' (2021) 1 *The Journal of Philosophy of Disability* 57, 65–6.

[77] Bach and Kerzner (n 42) 7, 12, 38.

[78] Ibid 38–44. For the concepts of 'negative' and 'positive liberty', Bach and Kerzner refer to Isaiah Berlin, 'Two Concepts of Liberty' in Henry Hardy (ed), *Liberty Incorporating Four Essays on Liberty* (Oxford Scholarship Online, 2002) 166, 170, 178–81. However, Kong's view is that their use of positive liberty in this way is a misinterpretation and misapplication of Berlin's theory. See: Kong, 'The Convention on the Rights of Persons with Disabilities and Article 12' (n 21) 720.

[79] Bach and Kerzner (n 42) 40–1.

Bach and Kerzner seek to resolve this contradiction by emphasising that Article 12(3) requires just the right number of supports necessary for a person to make an autonomous decision and exercise their legal capacity – no more, no less.[80] These supports are interpreted very narrowly and insularly as only those 'goods and services' directly required for the making of a particular decision. They include life-planning supports, independent advocacy, communication and interpretive supports, relationship-building supports and administrative supports.[81] Some of these Article 12(3) supports, especially life-planning supports, are potentially extensive and would in all practicality be required continuously to be effective. But according to Bach and Kerzner's logic, it is as if Article 12(3) only engages them at the point of making a decision. There is a reluctance to acknowledge the principle of indivisibility as being the interdependence between Article 12 and those other rights in the CRPD that may demand and claim ongoing socio-economic supports. It is as if personhood remains bounded within Article 12 and individual autonomy.

4.5 *Regulation as 'Positive Liberty'?*

Article 12(4) also requires positive action by the State in terms of regulation to guard against abuse, and to 'ensure that measures relating to the exercise of legal capacity respect the rights, will and preferences of the person'.[82] As noted in Chapter 2, while Article 12(4) has been interpreted as allowing for substitute decision-making, it also requires the integrity of supported decision-making to be protected.[83] Bach and Kerzner describe some state interventions as upholding 'positive' liberty or autonomy. *General Comment No 1* provides that legal recognition of support persons must be available, as well as a 'mechanism for third parties to verify the identity of a support person' and 'a mechanism for third parties to challenge the action of a support person'.[84] In their trusteeship model, Francis and Silvers also propose procedural protections against abuse to 'respond to people's differences without disadvantaging them for being so'.[85] While such interventions

[80] Ibid 95.

[81] Ibid 73–6.

[82] See also: 'Given the importance of safeguarding in the context of support, it is vital that supporters, particularly those who support financial decisions, are subject to appropriate levels of supervision by a public body'. Rosie Harding, 'Supporting Everyday Legal Capacity: Navigating the Complexities of Putting Rights into Practice' in Mary Donnelly, Rosie Harding and Ezgi Tascioglu (eds), *Supporting Legal Capacity in Socio-Legal Context* (Hart Publishing, 2022) 291, 312.

[83] See, e.g., Bartlett who writes of Article 12(4) that it 'refers expressly to the possibility that the exercise of legal capacity may be curtailed in some circumstances'. Peter Bartlett, 'The United Nations Convention on the Rights of Persons with Disabilities and Mental Health Law' (2012) 75(5) *The Modern Law Review* 752, 765. On the other hand, see *General Comment No 1* [20]–[22].

[84] *General Comment No 1* [29(d)].

[85] Silvers and Francis (n 68) 66.

are necessary to protect the integrity of the decision-making process, the UN Disability Committee prescribes that such regimes should nevertheless not 'over-regulate' the lives of people with disability.[86]

There are assumptions made about what kind of positive regulatory actions would be autonomy-enhancing and those that would be autonomy-denying. State regulation (or interference) designed to protect the integrity of supported decision-making, and therefore enhance autonomy, is framed as inhering within it a concept of positive liberty or autonomy. On the other hand, decision-making by substitutes is always autonomy-denying.

4.6 Why Refashioning Autonomy as Interdependence, Relational Autonomy or Relationships of Trust Does Not Work as Universal Personhood

Under the above accounts, an autonomous person is ostensibly not a bounded individual, but their personhood is constituted by their interdependence and is 'shared' because the supporter contributes to their 'positive liberty'. Relational autonomy is an umbrella term used by advocates of universal legal capacity to describe many of these variations of a theory of shared personhood. The boundaries around individuals as negotiators in the social contract are ostensibly reconfigured as lines of connection, not division.[87] External interference in decision-making is reframed as positive support so that the social contract and our personhood are recognised through the trustworthy honouring of our relationships. The supporter as cognitive prosthesis is one configuration of, or metaphor for, this notion of personhood as interdependent or shared.

4.6.1 How Does Alex's Situation Reveal the Problems with Retaining Refashioned Autonomy as the Basis of Personhood?

In the case of Alex, the supporter is not separate from Alex but is part of Alex's interdependent or shared personhood. This framing means that the supporter's critical role in Alex's personal life is autonomy-enhancing and balances positive liberty against negative. Under the trusteeship model, the relationship between Alex and the supporter is one of trust and connection. Alternatively, under the prosthesis model, it is as if Alex's bounded individuality is still intact, albeit the boundary has moved to include the rational 'prosthesis'.

If the supporter is trustworthy, he or she may well respect the preference (or will) of Alex to refuse dental treatment, given the strength of Alex's objections. Alternatively,

[86] *General Comment No 1* [29].

[87] Note Beaudry's argument and conclusion that Francis and Silvers's attempt to redefine the social contract as being constituted by relationships of trust ultimately fails to include all adults with severe and profound cognitive disability: Jonas-Sébastien Beaudry, *The Disabled Contract: Severe Intellectual Disability, Justice and Morality* (Cambridge University Press, 2021) 50.

if the supporter is contributing the rational component of the decision-making process by way of a prosthesis, he or she may attribute a more considered decision to Alex, as reflected in his longer-term desire to be able to eat gourmet food. So, even in the case of a trusted supporter, the decision may still go either way – but in either case, it will still be attributed to Alex.

This refashioning of autonomy to underpin Alex's personhood still raises problems about the certainty, transparency and integrity of supported decision-making. The following sections consider what these problems are and ultimately reject the refashionings of autonomy described above as a way of potentially recognising universal personhood. The chapter argues that: (1) shared personhood still does not tell us who is making the decision, (2) relational autonomy is not autonomy at all, (3) 'positive' liberty may lead to less autonomy (not more), (4) rationality and bounded personhood are still privileged and (5) the residual impacts of cognitive impairment on decision-making are still not acknowledged.

4.6.2 Shared Personhood Does Not Tell Us Who Is Making the Decision, Potentially Hiding Undue Influence or Abuse

Constructing Article 12 autonomy as interdependence or shared personhood fails to address one of the central challenges in supported decision-making. Supported decision-making, in shrouding or obfuscating the locus of decision-making power, can place the adult at risk of undue influence and exploitation. This is particularly a risk in the case of informal supported decision-making undertaken in the private sphere without state scrutiny, where the nature of the relationship between adult and supporter may be unclear.[88] In many cases, the concept of shared personhood may serve to hide who is truly making the decision; and it may expose adults to abuse and exploitation by supporters who are trusted family or personal friends.[89]

The theories of supporter as trustee or prosthesis face similar challenges in determining where the adult's personhood ends and the trustee's (or prosthesis's) begins. This is especially so because, according to this account of autonomy, the prostheses would appear to have no personhood themselves but are positioned as merely instrumental to the personhood of the adult. Del Villar criticises *General Comment No 1* for potentially treating 'the family instrumentally and not relationally' so that 'support is provided to enable a person with disabilities to act as an independent individual, not as a member of a family or community'.[90] The damage in viewing a supporter in this way, as instrumental to the personhood of the adult,

[88] Malcolm Parker, 'Getting the Balance Right: Conceptual Considerations Concerning Legal Capacity and Supported Decision-Making' (2016) 13(3) *Journal of Bioethical Inquiry* 381, 388.

[89] Anita Smith, 'Are Guardianship Laws and Practices Consistent with Human Rights Instruments?' in A Kimberly Dayton (ed), *Comparative Perspectives on Adult Guardianship* (Carolina Academic Press, 2014) 247, 266.

[90] Del Villar (n 39) 194.

is best illustrated in concrete terms by the comments of a supporter in a decision-making trial who reflected that:

> 'I'm just getting so drained, physically and emotionally, that I just feel like I'm shutting down, and I'm struggling to make decisions now, because I have to switch off, emotionally, so that I don't just fall apart and I'm not going to have time to not be productive.'[91]

Beaudry further criticises the trusteeship model for its failure to sidestep the problem that the trustee is 'a kind of delegate who would occupy a more active or intrusive role than delegates for non-disabled citizens or contractees'.[92] This suggests that the trustee is no more than a substitute decision-maker who thereby signals the adult's non-membership in the social contract and associated denial of personhood.

These accounts of shared or interdependent personhood do not explain how or when a supporter or a web of influence may be coercive and lead to denial of the 'individual autonomy' of the adult. Francis and Silvers acknowledge that their trusteeship theory cannot 'offer a full account of how justice through trust conceptualises exploitation'.[93] Simplican recognises this omission when she criticises the trusteeship theory for what she describes as its unsupported assumption that the 'non-disabled possess a high level of concern for the quality of treatment shown to people with severe disabilities … in a society which we know to be infused with ableist prejudices'.[94] Further, supporters as internal cognitive prostheses risk losing their own personhood as their agency is recognised only as far as it is instrumental to the personhood of the adult.[95]

4.6.3 Relational Autonomy: Different Kinds of Relationships

Writers have criticised the way that relational autonomy has been used by some of the above commentators to justify a conception of universal autonomy and legal capacity, and I agree with these criticisms. The concept of relational autonomy is useful in the context of challenging the '…notion of autonomy [as] fundamentally individualistic and rationalistic'[96] and in bringing attention to the social determinants of autonomy and the social embeddedness of our selves. However I challenge the logic of asserting that, while adults with severe and profound cognitive disability do not have autonomy in the traditional sense, they nevertheless possess relational autonomy and can therefore be recognised as having decision-making ability and legal capacity in all cases.

[91] Carney et al (n 33) 367.
[92] Beaudry (n 87) 172.
[93] Silvers and Francis (n 68) 65.
[94] Simplican (n 72) 94.
[95] Del Villar (n 39) 194.
[96] Mackenzie and Stoljar (n 15) 3.

Nedelsky, a prominent theorist of relational autonomy, explains the concept as a theory that relationships are constitutive of the self, but also that:

> autonomy is made possible by constructive relationship (or undermined by destructive relationship) … Autonomy can thrive or wither in adults depending on the structures of relationship they are embedded in.[97]

For Nedelsky, under a relational account, autonomy is never presumed[98] and never universal. She does, however, subscribe to the idea that everyone has a capacity for autonomy, even if it cannot be achieved without the *right* supports and relationships.[99] Nedelsky uses relational autonomy as an analytic tool to show how relationships operate in society on both the interpersonal and structural levels to foster individual liberty and security.[100]

Criticisms of the way that relational autonomy has been used to explain Article 12 autonomy echo those above that criticise that 'Facilitated decision making stretches the concept of RA to its limits'. Martin et al write that relational autonomy on a 'radical interpretation' would mean that 'a person could be deemed autonomous despite lacking decision-making ability', which would be 'an abuse of language (i.e. of the language of autonomy)'.[101] Ho also raises the double-edged sword of that 'radical' conception of relational autonomy, which is that the supporter may not be benign but abusive.[102] Wilson similarly writes that the use of relational autonomy to justify or explain universal legal capacity 'misunderstands the concept' which she explains as referring to '…how a person's relations in a wider sense with other people, the state, law and the world, support or hinder an individual's autonomy and how law structures and nurtures those relationships'.[103]

I agree with these authors that using relational autonomy to explain and justify recognising universal legal capacity exhibits a narrow understanding and application of the concept. Relational autonomy ultimately retains the distinction between those adults who have autonomy and those who do not but serves as a tool to investigate which relationships and social situations enhance autonomy and which undermine it.[104]

[97] Jennifer Nedelsky, *Law's Relations: A Relational Theory of Self, Autonomy, and the Law* (Oxford University Press, 2012) 39.

[98] Ibid 278.

[99] Ibid 46.

[100] Ibid 90.

[101] Wayne Martin et al, *Achieving CRPD Compliance: Is the Mental Capacity Act of England and Wales Compatible with the UN Convention on the Rights of Persons with Disabilities? If Not, What Next?* (Essex Autonomy Project, 2014) 25–6.

[102] Anita Ho, 'Relational Autonomy or Undue Pressure? Family's Role in Medical Decision-Making' (2008) 22(1) *Scandinavian Journal of Caring Sciences* 128, 128–9.

[103] Kay Wilson, *Mental Health Law: Abolish or Reform?* (Oxford University Press, 2021) 159.

[104] Kong refers to a 'relational analysis of rights' as being required to understand and make transparent the impact of social interactions and values (including the internalisation of those values) on the individual's autonomy and decision-making processes: Kong, *Mental Capacity in Relationship* (n 46) 43; Also, Hall describes relational autonomy as providing for a more transparent and accurate

Relational autonomy as conceptualised by Mackenzie and Stoljar, Nedelsky and others[105] values autonomy but also recognises that those without autonomy need to be supported with the right kinds of supports to achieve it.

4.6.4 'Positive' Liberty Includes Regulatory Intervention by the State

Bach and Kerzner refashion the concept of autonomy from negative liberty or restraint from interference by the State to a balancing of negative and positive liberties – that is, state restraint against state action.[106] State action includes safeguards such as the registration and regulation of supporters to further preserve and support autonomy.

Existing and proposed regulatory models for supported decision-making include the same types of regulatory mechanisms as Australian guardianship regimes but with added components to deal with different complexities. They may allow for registration of supporters; establishment of a state regulatory agency and a tribunal; registration of support agreements; imposition of fees; statutory duties for supporters; access by supporters to otherwise confidential information; immunity for supporters acting in good faith; appointment of a 'monitor' to check the work of the supporter; state investigation of supporters acting contrary to law and de-registration of a support person who is acting improperly or illegally.[107] While registration of support persons would not be mandatory, the potential for a registration system to have a 'nudge' effect towards delegitimising the role of unregistered supporters would, I suggest, be strong and could undermine its voluntariness. I argue that there is a very real question as to whether such systems would undermine the autonomy of the adult any more than would a best-practice guardianship system. While other commentators have similarly admonished against overregulation, there has been no agreement on the balance to be struck.[108]

lens through which to view the way that subjectivity, personhood and decision-making actually operate for individuals. She describes how: 'Relationships per se are not sufficient for the development of autonomy in the relational account. The **right kind** of relationship context …, is required' in Margaret Isabel Hall, 'Mental Capacity in the (Civil) Law: Capacity, Autonomy, and Vulnerability' (2012) 58(1) *McGill Law Journal – Revue de Droit de McGill* 61, 87.

[105] See also: Linda Barclay, 'Autonomy and the Social Self' in Catriona Mackenzie and Natalie Stoljar (eds), *Relational Autonomy: Feminist Perspectives on Autonomy, Agency and the Social Self* (Oxford University Press, 2000) 52; Marilyn Friedman, 'Autonomy, Social Disruption and Women' in Catrionia Mackenzie and Natalie Stoljar (eds), *Relational Autonomy: Feminist Perspectives on Autonomy, Agency and the Social Self* (Oxford University Press, 2000) 35.

[106] Bach and Kerzner (n 42) 95.

[107] For a description of the legislation in the Canadian provinces, Ireland and Victoria, see: Julia Duffy and Kelly Unsworth, *Supported Decision-making, Legal Risk and Commercial Uncertainty* (17–18 October) (Australian Guardianship and Administration Council 2016 National Conference: Reflecting Will and Preference in Decision-making) 5–9.

[108] Arstein-Kerslake, 'An Empowering Dependency' (n 29) 88; of the MCA and the safeguards required under Article 12(4), Bartlett writes that it is 'difficult to say what processes would be appropriate, that would not force the system to collapse under its own administrative weight': In Bartlett (n 83) 768.

Carney has repeatedly warned against the 'net-widening' effect of formal supported decision-making systems.[109] There is a potential, if not inevitability, that if supported decision-making were widely formalised, more adults with disability would find themselves subject to state regulation than under existing guardianship regimes. And yet, if there is no formalisation, the private sphere of supported decision-making would lack transparency and accountability, and the locus of decision-making power would be obscured.[110]

4.6.5 Failure to Acknowledge the Residual Impacts of Impairment

As Plumb writes, 'Autonomy (self-determination) is fundamental to the social model of disability.' For her, the concepts are well aligned for adults with *physical* disability, whose agency can be exercised once accommodated for and recognised by people around them.[111] However, a notion of interdependent personhood or relational autonomy for adults with severe and profound cognitive disability fails to recognise the residual impacts of impairment after support is provided. The hard reality is that cognitive impairment can limit the capacity for reasoning and decision-making. Further, to universalise and normalise personhood as interdependence fails to recognise that dependencies may be different and greater for those with cognitive disability than for others and that this may make them more vulnerable to abuse and exploitation.[112] Chapter 3 has already described how impairment can be associated with a cycle of socio-economic disadvantage, and that people with disability are disproportionately poor compared with the wider population. Paradoxically, the higher levels of dependency associated with cognitive impairment are acknowledged in the consensus that support for decision-making is required. Yet that acknowledgement is paradoxically pushed aside or forgotten in the determination to retain at all

[109] Terry Carney, 'Participation and Service Access Rights for People with Intellectual Disability: A Role for Law?' (2013) 38 *Journal of Intellectual and Developmental Disability* 59, 62; Carney, 'Supported Decision-Making for People with Cognitive Impairments: An Australian Perspective?' (n 125) 41; Terry Carney, 'Supporting People with Cognitive Disability with Decision-Making: Any Australian Law Reform Contributions?' (2015) 2 *Research and Practice in Intellectual and Developmental Disabilities* 6, 10; Terry Carney and Fleur Beaupert, 'Public and Private Bricolage: Challenges Balancing Law, Services and Civil Society in Advancing the CRPD Supported Decision Making' (2013) 36(1) *University of New South Wales Law Journal* 175, 195.

[110] 'While informality advances autonomy through compliance with statutory least restrictive alternative principles … it comes at the cost of enhanced opportunities for abuse and/or misreading of wishes' in Carney and Beaupert (n 109) 195.

[111] Anne Plumb, 'UN Convention on the Rights of Persons with Disabilities: Out of the Frying Pan into the Fire? Mental Health Service Users and Survivors Aligning with the Disability Movement' in Helen Spandler et al (eds), *Madness, Distress and the Politics of Disablement* (Bristol University Press, 2015) 183, 186–7.

[112] *General Comment No 1* [22] recognises this increased vulnerability: 'All people risk being subject to "undue influence", yet this may be exacerbated for those who rely on the support of others to make decisions.'

costs liberal autonomy as the foundation of personhood. I argue that retention of autonomy as the basis for personhood is underpinned by a strict version of the social model that fails to acknowledge the residual impacts of cognitive impairment and associated disadvantages.

4.6.6 Refashioning Autonomy Retains the Privileging of Rationality (but Just Relocates It in the Supporter)

Finally, I argue that despite a disavowal of the privileging of rationality as the foundation of autonomy, autonomy as interdependence retains that privileging, merely relocating the rational element into the supporter. Series criticises Silvers and Francis's prosthetic model of decision-making because it erroneously proposes that the internal processes of decision-making can be outsourced to a third party while the decision remains the adult's own.[113] Under the prosthesis theory, 'supporters should be viewed as a prosthesis for thinking, whereby their reasoning and communication can execute part or all of a subject's own thinking processes'.[114]

Building on Series's critique, I argue that the prosthesis theory also serves to retain the Kantian privileging of rationality, but that it relocates the reasoning function into the prosthesis as supporter. Indeed, taking a broader critique, I argue that retaining autonomy as the basis of interdependent personhood risks stigmatising the adult with cognitive impairment who lacks any reasoning capability (i.e. without the help of a supporter/prosthesis). If a supporter is executing all, or even part, of a subject's thinking processes, would not transparency and accountability be better served by describing and acknowledging the outcome as a decision by a substitute?

4.7 *Returning to the Case of Alex*

In the case of Alex, we saw above that under a supported decision-making model, his decision could be interpreted as either agreeing to the dental treatment or refusing it. The outcome would depend on which view of autonomy we adopt, how we interpret Alex's life values, or how we construct his life as a coherent narrative. Autonomy reformulated as interdependence, as a relationship of trust, or as being supplied by a cognitive prosthesis takes the analysis no further. Indeed, if trust is to be wrongly assumed, Alex's autonomy may be undermined by a supporter's own reasons for refusal of dental treatment. The supporter may be a family member or worker who thought it would be easier and cheaper to care for Alex if he lost all of his teeth, didn't need high-quality food, and didn't have to be taken on so many outings. Such a decision would accord with Alex's expressed wish not to have the dental treatment, but it is not clear that it would further his autonomous personhood.

[113] Series (n 41) 86.
[114] Ibid.

If we acknowledge the impacts of impairment on Alex after all supports are provided, then we can acknowledge his personal and situational disadvantage in the decision-making situation. There are reasonable views on either side as to what is the 'right' decision in this case. But if Alex foregoes the treatment and later loses his teeth, he will ultimately feel unhappy with the resulting loss of enjoyment. This raises a legitimate ethical issue around attributing the decision to Alex. In an ableist world where many people may think that such joys could not be truly appreciated by someone with a disability, this interpretation may be perceived as placing a lesser value on Alex's life. If a substitute were to make the decision, the locus of decision-making power would be clear. If the dentist or other person felt uneasy about the decision to either forego or pursue treatment, best-practice substitute decision-making would include an independent pathway for a challenge that would take Alex's expressed views into account.

4.8 Summary and Conclusions on Refashioning Autonomy to Underpin Universal Legal Capacity and Inclusive Personhood

Personhood, as derived from Kant finds its foundations in autonomy as rationality and independence, thereby excluding adults with cognitive disability. To include adults with cognitive disability within conceptions of personhood, either: (1) the liberal conception of autonomy must be reformulated or (2) alternative bases for personhood must be recognised. Those commentators advocating an interpretation of Article 12 as requiring recognition of universal legal capacity and the abolition of decision-making by substitutes have followed the first strategy.

According to this first strategy, writers have refashioned autonomy as interdependence and shared personhood or relational autonomy, endeavouring to destigmatise and normalise the dependencies experienced by adults with cognitive disability, particularly under the supported decision-making model. Some theories suggest that a Rawlsian bargaining paradigm should be reconfigured as relationships of trust, or that the supporter be reconceptualised as the adult's cognitive prosthesis. To account for the role of the State and others in providing supported decision-making, Bach and Kerzner refashion Millian liberty so that it incorporates positive as well as negative liberty.

Relational autonomy has been relied on to recognise the positive impacts of a trusted supporter on an adult's autonomy. However, I argue that relational autonomy used in this way ultimately reinforces the liberal account of autonomy, as attached to and located in the personhood of the adult as an individual. More broadly, the problem with autonomy as interdependence is that the individual risks becoming lost in a web of relationships in a way that makes it impossible to determine who is actually making the decision. Shared personhood and relational autonomy as interdependence rely on a strict social model of disability that locates disability entirely in the social environment, ignoring the residual impacts of cognitive impairment.

By way of comparison, a social model that recognises the residual impacts of impairment demands recognition of the difference of impairment/disability and the impact of that difference on autonomous decision-making, even after all the supports are provided (or when they are unavailable). Acknowledging the residual impacts of impairment also allows for acknowledgement that state provision and regulation of support services, even if reframed as providing 'positive liberty', may be no more autonomy-enhancing than guardianship used as a last resort. According to Kong, the 'will and preference' paradigm and supported decision-making purport to be value-neutral in the liberal tradition but determining what counts as a restraint is value-laden.[115] Support is valued as promoting autonomy and human rights, even though it acknowledges dependency, operates in the personal sphere and attracts regulatory oversight. Decision-making by substitutes, on the other hand, is framed in all cases as perpetuating dependencies, interfering in private lives and undermining autonomy and personhood.

I argue that in all of the above refashionings of autonomy, which aim to retain its privileged position as the foundation of personhood, there are unresolved contradictions. Despite assertions to the contrary, I argue that they still privilege rationality (albeit located in the supporter/prosthesis) in a way that risks perpetuating stigmatisation and exclusion of adults with cognitive impairment. I further argue that under these refashionings, the adult's personhood is still bounded. This is because so-called interdependencies, shared personhoods, or relational autonomies (under these accounts) fail to recognise the autonomy and personhood of the supporter – who is actually described by one theory as a 'prosthesis' that is instrumental to the personhood of the adult. I would argue that the individual is still bounded in the Kantian tradition. It is just that the boundaries have moved to include as part of the individual's autonomous personhood certain carefully defined supports that the State must provide. Bach and Kerzner draw a boundary between the supports provided under Article 12(3) and those socio-economic supports that, while assisting with legal capacity and decision-making, are required separately under other articles of the CRPD. Bach and Kerzner acknowledge that provision of 'financial resources, housing and disability related supports etc. all affect the exercise of legal capacity'[116] in that, with fewer resources and supports, a person has correspondingly fewer choices. Nevertheless, they draw a boundary between the right in Article 12 to supports and the right to claim this more extensive range of financial resources and supports. According to this account, autonomy as personhood incorporates supports only to the extent that they directly assist with a particular decision. This account thereby fails to recognise the interdependency between the right in Article 12 to decision-making with supports and other rights in the convention.

[115] Kong, *Mental Capacity in Relationship* (n 46) 36.
[116] Bach and Kerzner (n 42) 73.

Universal autonomy as the basis for personhood does nothing to disrupt the liberal paradigm and risks perpetuating the marginalisation of adults with severe and profound cognitive disability who do not have autonomous decision-making capacity. Hall suggests: 'the ideological paradigm in which the capacity construct is situated and of which it is an essential part – the liberal autonomy idea – is itself, in this context, deeply problematic and even cruel'.[117] I argue that it is indeed cruel because it continues to privilege autonomy as the basis of personhood, inevitably excluding some adults with cognitive disability.

The case of Alex shows that recognising autonomy universally fails to provide certainty or transparency and does not always protect an adult's rights.[118] If Alex foregoes dental treatment, then his socio-economic rights to health and social inclusion will be compromised – and possibly his longevity. Such an outcome – whether reached by supported or substitute decision-making – could be premised on unspoken ableist assumptions that health, welfare, social inclusion and long life are not as critically important to Alex as to someone without a disability. If a substitute decision were made to have the treatment, Alex's Article 16 CRPD right to physical integrity may be compromised, but the impacts could be ameliorated by medication and the presence of a support animal. The answers in situations such as these are not easy, but the case study demonstrates that supported decision-making practices that purport to recognise autonomy in adults with compromised decision-making abilities bring their own ethical and legal problems. I further argue that, by not recognising the interdependency of Article 12 with other rights in the CRPD and the equal importance of socio-economic rights (especially in the case of disability), they fail to fully apply the principle of indivisibility of human rights.

5 DIVORCING AUTONOMY FROM PERSONHOOD: BUT STILL REFASHIONING AUTONOMY TO BE MORE INCLUSIVE

This section describes those refashionings of autonomy that align with the second strategy – that is, valuing autonomy as one among many worthy social goods but divorcing it from personhood. This strategy allows for or demands an alternative foundation for personhood inclusive of adults with cognitive disability.

I conclude that Article 12 autonomy is best aligned with those conceptions of autonomy that:

(1) conceive of autonomy as achievement or as the development of autonomy competencies; and
(2) demand a wide range of meaningful options from which to choose.

[117] Hall, 'Mental Capacity in the (Civil) Law' (n 104) 64.
[118] As Foster writes: 'it is impossible to know when a truly autonomous decision has been made, or what conditions must pertain for it to be made'. Foster, *Choosing Life, Choosing Death: The Tyranny of Autonomy in Medical Ethics and Law* (Hart Publishing, 2009) 13.

These two conceptions of autonomy require the fulfilment of a wide range of socio-economic rights.

At the end of this section, I return to the case of Alex to illustrate how autonomy as achievement, as the development of competencies and as demanding real options applies in his case. Section 6 will explain how this conception of Article 12 autonomy embodies the indivisibility of human rights.

5.1 *Autonomy as Achievement*

Donnelly reconceives autonomy as a process of achievement and also attempts to dislodge the absolute nexus between autonomy and capacity. Before the publication of *General Comment No 1*, she expressed the view that Article 12(2) CRPD was 'unlikely' to be interpreted as a prohibition on all capacity-based assessments.[119] Donnelly works from within the liberal tradition but is less determined than others to adhere to a privileging of autonomy over other rights and values, concluding that 'autonomy, capacity and incapacity are less important than under the traditional liberal view'.[120] She invokes Raz, who conceived of autonomy as 'a kind of achievement'[121] or empowerment and applies this conception of autonomy to the law and practice of health-care decision-making. She is, in particular, critical of the 'simple view of autonomy as non-interference [that] still pervades much legal discourse'.[122] Donnelly writes that:

> respect for autonomy requires more than non-interference. Instead, a legal framework for autonomy must be concerned with issues such as the adequacy of choice and the development of autonomy-building measures aimed at enhancing individual agency and a shift in decision-making power to the individual. This would seem to be the view of autonomy underlying the CRPD…. The aim is to empower individuals and to support them in making decisions for themselves regardless of capacity.[123]

Like Quinn and Arstein-Kerslake, and Bach and Kerzner, Donnelly impliedly incorporates the requirement to provide support for decision-making (i.e. 'autonomy-building measures') into her view of autonomy. However, the term 'autonomy-building measures' is much wider than the term 'support'. I suggest it envisages a more holistic and ongoing dependency on positive state measures than those that may apply to the immediate circumstances surrounding a specific decision. Donnelly is critical of the liberal account of capacity and autonomy for ignoring the impact of external social and environmental factors on decision-making capacity.[124]

[119] Donnelly (n 2) 111.
[120] Ibid 157.
[121] Joseph Raz, *The Morality of Freedom* (Clarendon Press 1986) 204.
[122] Donnelly (n 2) 23.
[123] Ibid 257.
[124] Ibid 7.

She proposes that an adult with cognitive disability who engages in 'participative' decision-making may have their autonomy respected while nevertheless being found to lack capacity. Ultimately, though, Donnelly unambiguously recognises that some adults will have neither autonomy nor capacity due to their cognitive disability and suggests using dignity as a value to interpret human rights would be more inclusive than autonomy.[125]

5.2 *Autonomy as the Development of Autonomy Competencies*

Similar to Donnelly's idea of autonomy as achievement are theories that propose autonomy competencies can be either developed or neglected.[126] In referring to 'autonomy competencies', these theories articulate a substantive concept of autonomy because they conceive it as more than a prescribed procedure (e.g. independence and non-interference) and value some attributes or accomplishments over others.[127] Inherent in the notion of autonomy competencies is also the idea that autonomy is something that, in many cases, can and must be achieved through positive actions; it is not simply an absence of restraints. Dodds writes that:

> it seems clear that in the provision of basic welfare services (health, education, housing, etc.) a society can promote personal autonomy by enabling access to those things needed to develop autonomy competency and to pursue an autonomous life. Health-care systems are clearly part of this provisioning, insofar as good health and decent access to health-care facilities will be a precondition for the pursuit of a very greater range of autonomous projects.[128]

Kong criticises the UN Disability Committee for its view that protection is always inconsistent with autonomy and for failing to see autonomy as one of *many* goods that need to be valued.[129] In particular, Kong points to the right to education in Article 24 CRPD as one social good that fosters autonomy competencies.[130] She endorses a scalar view of autonomy and argues that autonomy can be effected through engaging in particular types of relationships that foster the perceptual and self-reflective skills essential to achieving autonomy.[131] The idea of autonomy as a competency allows us to frame decision-making by substitutes as potentially one tool to promote or achieve autonomy.

[125] Ibid 212.
[126] Christman, 'Constructing the Inner Citadel' (n 6) 115; Mackenzie and Stoljar (n 15) 56, 240; Kong, *Mental Capacity in Relationship* (n 46) 147.
[127] Ashley (n 18) 17. See also: 'people must have autonomy competency – the repertory of coordinated skills that makes self-discovery, self-definition and self-direction possible'. Meyers (n 32) 76.
[128] Susan Dodds, 'Choice and Control in Feminist Bioethics' in Catriona Mackenzie and Natalie Stoljar (eds), *Relational Autonomy: Feminist Perspectives on Autonomy, Agency and the Social Self* (Oxford University Press, 2000) 213, 228.
[129] Kong, *Mental Capacity in Relationship* (n 46) 46–7.
[130] Ibid 47.
[131] Ibid 98.

5.3 *Autonomy Necessitates the Provision of Real Choices and Options*

Donnelly and Dodds also consider how autonomy necessitates, or inheres within it, a demand for real options. Donnelly rejects the traditional view of autonomy as being effectively limited to the right to either consent to or, more markedly, refuse treatment.[132] Her concept of autonomy as achievement means that autonomy itself places a demand on others – health-care providers, practitioners and the State – to offer alternatives as well as to provide education and to 'create a place for a decision'.[133] Respect for autonomy as achievement and empowerment demands that the State ensure a range of alternative services. She quotes Raz as saying that 'the provision of many collective goods is constitutive of the very possibility of autonomy'.[134]

Dodds makes similar arguments about autonomy in the health-care setting.[135] She goes further to critique the dominant proceduralist view of autonomy as 'mere choice' by pointing out that how health-care choices are presented to a patient will depend on many social and environmental factors. These will include the range of available resources, institutional frameworks and the nature of health-practitioner education.[136] For example, it may be that in the mental health-care setting, it is not always the involuntariness of the treatment that is problematic but the nature of the treatment being offered and the associated lack of options.[137] As Raz writes:

> Another aspect of autonomy concerns the quality of the options open to agents. The autonomous agent is not always struggling to maintain the minimum conditions of a worthwhile life. The more one's choices are dictated by personal needs, the less autonomous one becomes.[138]

5.4 *Returning to the Case of Alex*

In the case of Alex, he is fortunate to have the option of affordable dental treatment. If affordable dental services were beyond his reach, no decision would need to be made and his legal capacity would not be compromised by the intervention of

[132] Donnelly (n 2) 39.
[133] Ibid.
[134] Ibid 42.
[135] Mackenzie and Stoljar (n 15) 25.
[136] Dodds, 'Choice and Control in Feminist Bioethics' (n 128) 223.
[137] Plumb opines that involuntary treatment is sometimes needed to keep adults safe from self-harm, but that when it comes to designing such treatment, health professionals need to be able to offer services that the adult actually feels meet their own criteria for safety: Plumb (n 111) 190.
[138] Raz (n 121) 155; see also: 'The self-governing individual must have access to an adequate assortment of options' in Oshana (n 26) 94.

either a supporter or substitute. However, if there were no dental services available, it would be hard to argue against the fact that Alex would be making the 'decision' to forego treatment from a position of relative disempowerment. Raz, Donnelly and others would argue that Alex's autonomy has been compromised not only through cognitive impairment but through a lack of alternatives, thereby recognising the reality of his disempowered position.

5.5 *Commentary and Conclusions*

Article 12, with its right to support for legal capacity, eschews a concept of autonomy as the right to be left alone and to make choices without the help of others. Donnelly conceives of Article 12 autonomy, as achievement or empowerment. She recognises that even adults who may not meet tests for decision-making capacity may still have their autonomy respected in participating in the process of decision-making. Others view autonomy in a similar vein, as a process of developing certain competencies to enable self-determination in decision-making. Dodds and Kong emphasise the provision of adequate welfare services including health, education and housing as pre-conditions for an adult to undertake autonomous pursuits.[139]

Article 12 recognises the role of other people and services in the achievement of an adult's autonomy or the development of their autonomy competencies. In doing so, Article 12 demands that the State take action in the personal realm to promote autonomy. Reformulations of autonomy as achievement or as the development of autonomy competencies, and as also demanding a range of acceptable options, are consistent with an Article 12 conception of autonomy that includes a right to support for decision-making. However, a conception of autonomy as achievement, and as incorporating a demand for a range of options, indicates not just interdependence of rights within Article 12 itself (i.e. the right to support and the right to legal capacity) but interdependence between Article 12 and other rights in the CRPD, especially socio-economic rights. Section 6 considers the implications of a conception of autonomy as achievement and as demanding a range of options for the indivisibility of human rights in the CRPD.

6 AUTONOMY AND THE INDIVISIBILITY OF HUMAN RIGHTS

Chapter 3 described how a significant element of the principle of indivisibility is the elimination of the hierarchy of rights: civil–political over socio-economic.

[139] 'it seems clear that in the provision of welfare services (health, education, housing, etc.) a society can promote personal autonomy by enabling access to those things needed to develop autonomy competency and to pursue an autonomous life', in: Dodds, 'Choice and Control in Feminist Bioethics' (n 128) 228; Kong refers to education: Kong, *Mental Capacity in Relationship* (n 46) 46–7.

Linked with this is the recognition that the association of certain indicia with either one category or the other is overstated or misplaced. The features typically attributed to civil–political rights are that they impose negative state obligations, can be implemented immediately, are cost-free, protect the individual from the State and are justiciable. The attributes of socio-economic rights are the converse: they impose positive state obligations, will be implemented progressively, are resource-intensive, operate in the public and private spheres and are non-justiciable. The UN Disability Committee has categorised the whole of Article 12 as a civil-political right, but I concluded in that chapter that Article 12 is better described as a 'hybrid' right.[140] As such, it disrupts the traditional dichotomies by imposing negative obligations on the State – but also positive obligations through the provision of support.

I argue in this section that the reformulations of autonomy as achievement, as the development of autonomy competencies, and as demanding a range of options reflect the principle of indivisibility of human rights. These reformulations can be interpreted as acknowledging the interdependency of the Article 12 right to legal capacity with other rights, especially socio-economic rights, thereby affirming the equal importance of the two sets of rights. At the end of the section, I return for a final time to the case of Alex to illustrate how decision-making by a substitute may be required to uphold his indivisible human rights.

6.1 Autonomy as the Foundation of Civil-political Rights

Autonomy is traditionally regarded as the foundation of civil–political rights, including the right to legal capacity to make decisions.[141] The European Court of Human Rights holds it as foundational to the right to privacy and family life,[142] thereby reflecting the concept of autonomy as sovereignty over one's personal sphere to be enjoyed without interference. Civil-political rights are traditionally marked as requiring restraint or negative action by the State, aligning with respect for liberal autonomy as requiring non-interference. Chapter 3 described how civil-political rights were originally framed in the ICCPR largely in terms of non-interference.[143] Behind these proscriptions are assumptions that as long as the State respects autonomy by remaining at arm's length, the default position will be

[140] See Chapter 3, Section 6.
[141] Ashley (n 18) 5.
[142] Foster (n 118) 32.
[143] For example, Article 7 *ICCPR* provides that, 'No one shall be subjected to torture or to cruel, inhuman or degrading treatment or punishment'; Article 9 that 'No one shall be deprived of his liberty except on such grounds and in accordance with such procedure as are established by law'; and Article 17 that 'No one shall be subjected to arbitrary or unlawful interference with his privacy, family, home or correspondence.'

that everyone has their rights realised. In Article 12 terms, the parallel assumption is that if the State refrains from interference in the form of guardianship, adults will have access to already available supports in the community and must not be supported against their wishes.[144]

6.2 *Autonomy Competencies and Socio-economic Rights*

The language used varies, but many of the reformulations of autonomy described above incorporate to a greater or lesser extent a concept of a right to autonomy as imposing positive obligations on the State and others. Autonomy refashioned as 'achievement' or 'empowerment', or as requiring the development of autonomy competencies, places positive obligations on the State to maximise autonomy by providing the goods and services needed to promote it. Such goods and services include immediate supports for decision-making but also the ongoing fulfilment of a wide range of socio-economic rights, including education, health, accommodation and social inclusion. In her book *Human Rights Transformed: Positive Rights and Positive Duties*, Fredman writes:

> People can only be fully autonomous if the State creates the conditions for valuable autonomy. The State's duty to promote personal autonomy requires it to create the conditions of autonomy. Crucially, this includes positively encouraging the flourishing of a plurality of incompatible and competing pursuits, projects, and relationships.[145]

DeBeco writes of the CRPD that 'the requirement to provide enabling conditions for exercise of human rights thus gives a socio-economic dimension to classical liberal rights'.[146] I argue more particularly that Article 12 demands a conception of autonomy that recognises its achievement may depend on fulfilling the range of socio-economic rights contained in the CRPD.

6.3 *A Range of Options Requires Fulfilment of a Range of Rights*

Donnelly above argues that autonomy demands a range of viable options from which an adult may choose; and, in the case of health care, not just the option

[144] Quinn refers to supports that 'occur naturally in the community' in Quinn (n 29) 10, 15; but there is 'a more muscular obligation' on the State to provide supports if an adult is in isolation. See: Quinn and Rekas-Rosalbo (n 29) 301. For the prohibition on providing supports against an adult's wishes, see *General Comment No 1* [19], [29(g)].

[145] Sandra Fredman, *Human Rights Transformed: Positive Rights and Positive Duties* (Oxford University Press, 2008) 24.

[146] Gauthier DeBeco, 'The Indivisibility of Human Rights and the Convention on the Rights of Persons with Disabilities' (2019) 68 (January) *International and Comparative Law Quarterly* 141; Michael Ashley Stein, 'Disability Human Rights' (2007) 95(1) *California Law Review* 75, 158.

of consenting to or refusing the treatment offered. Fredman similarly writes that 'autonomy is protected by placing a positive obligation on the State to provide a range of options for citizens to pursue, ensuring all have the capacity to pursue their valued options, and that all valued options are treated in the same way',[147] so that the emphasis in achieving autonomy and liberty moves 'from non-intervention to enhancing choice'.[148] She emphasises that constraints can arise as much from 'poverty, poor health, lack of education as from tyranny and intolerance' and that people need to be 'actually able to exercise their choices, rather than simply having a formal right to do so.[149]

Applying this to Article 12 decision-making, an adult with cognitive disability may be provided with the immediate supports to make a decision as to where to live. However, depending on the type of impairment and the supports required for daily living, such adults may be presented with very few suitable accommodation alternatives, if any at all. Unless the State expends resources on tailored accommodation with independent living supports, their only choice may be to live in inappropriate institutionalised accommodation that segregates them from family and the wider community and leads to diminished life skills and autonomy.[150] An abstract right to legal capacity or autonomy is impotent and meaningless without such a range of options. Fulfilment of socio-economic rights across a range of areas is a prerequisite for the exercise of an adult's legal capacity, whether decisions are made by the adult or by a substitute.

6.4 Disrupting the Boundaries between the Individual and the State: Private and Public Spheres

The liberal conception of autonomy traditionally draws a bright line between the personal and public realms, between the individual and the State. Fredman explains that freedom as non-interference characterises the State 'as separate from and opposed to the individual' and as 'an ever-increasing threat to individual autonomy',[151] and that recognition of the State's positive human rights obligations serves to challenge these assumptions. In the same vein, O'Cinneide writes that the interdependency recognised by the CRPD is not just between individuals but

[147] Fredman (n 145) 4.
[148] Ibid 10.
[149] Ibid 11.
[150] In Australia, this lack of funding for in-home resources has led to a large number of young adults with disability being inappropriately placed in aged-care homes. See: Australian Government, 'Younger People in Residential Aged Care – Action Plan', *Royal Commission into Violence, Abuse, Neglect and Exploitation of People with Disability* (undated), VIC.0001.0505.3677.pdf (royalcommission.gov.au).
[151] Fredman (n 145) 9.

between the individual and the State.[152] The CRPD acknowledges such an interdependency because it places obligations on the State to provide the social conditions necessary for a meaningful life.[153] O'Cinneide distinguishes the CRPD from a paternalistic model because it articulates social and economic measures as the fulfilment of individual rights, not as acts by the State of discretionary benevolence.[154] Similarly, Fredman explains that with social rights the emphasis is on public goods, which anticipates a rights-bearing citizen 'rather than a passive recipient of State welfare'.[155]

The UN Disability Committee views a loss of autonomy and legal capacity as the loss of claims to any and all rights. According to this logic, recognition that a person lacks autonomy inevitably results in services being provided under a charity or welfare model. I argue that even for those who lack autonomy, the principle of the indivisibility of human rights means that services can and must be claimed – whether under a supported or substitute decision-making model – as a matter of right.

6.5 *Indivisibility and the Case of Alex*

In the case of Alex, the development of his autonomy competencies depends on fulfilling a wide range of indivisible socio-economic rights in the CRPD – such as rights to health, education and appropriate accommodation. Let us assume, however, that he is among those disproportionate numbers of people with disability who grew up in poverty and were denied access to a range of socio-economic goods and services required to fulfil their socio-economic rights. Perhaps if Alex had been provided with better, tailored education and access to a range of health services from an early age, he would either not now have dental disease or would be more accustomed to and less intimidated by health professionals so as to be in a more informed position to make a decision on treatment.

Looking forward, if Alex were left to make a decision on his own to refuse dental treatment, his Article 12 legal capacity right may ostensibly be recognised, but he would be denied other human rights. Most notably, he would be denied the right to health care (Article 25), and his rights to social inclusion (Article 19) and participation in cultural life, recreation, leisure (Article 30) would be compromised.

[152] Colm O'Cinneide, 'Extracting Protection for the Rights of Persons with Disabilities from Human Rights Frameworks: Established Limits and New Possibilities' in Oddný Mjöll Arnadóttir and Gerard Quinn (eds), *The UN Convention on the Rights of Persons with Disabilities: European and Scandinavian Perspectives* (Nijhoff, 2009) 163, 172.

[153] Ibid 165.

[154] Ibid 165–6.

[155] Fredman (n 145) 235.

I argued in the earlier sections of this chapter that autonomy is not a satisfactory foundation for personhood because it excludes adults with cognitive disability. I further argued that autonomy cannot be successfully refashioned without discarding all of its original meaning. If another basis for personhood could be recognised and a substitute were appointed to make a decision on Alex's behalf, the appointment and decision would not designate Alex as a non-person. The substitute could make a decision in which Alex's whole range of rights was considered, and the substitute could thereby claim those rights for Alex, thus recognising that his personhood would be based on other, more inclusive grounds. Chapter 5 argues that the alternative ground of personhood relevant to the case of Alex, and all of us, is dignity.

7 SUMMARY AND CONCLUSION

Article 12 CRPD has been said to mark a paradigm shift in disability rights because it situates adults with disability as the subjects, not object, of rights and recognises autonomy as the basis of personhood in all cases. However, autonomy is founded on Kantian and Rawlsian rationality; on the ability to exercise will not just desire; on the ability to make considered decisions independently; and on the premise that action by the State and others in our private, personal sphere will limit our freedom and compromise our autonomy. Adults with cognitive disability have been traditionally excluded from Kantian personhood. This means that if autonomy is retained as the basis of personhood, the traditional concept of autonomy has to shift – or an alternative, more inclusive, basis for personhood has to be recognised.

One strategy to include adults with cognitive disability in personhood has been to refashion autonomy. The Article 12 literature has attempted to refashion autonomy from being constituted by rationality, independence and lack of interference to being marked by preference and emotion, interdependence, shared personhood and positive liberty. Autonomy as interdependence, shared personhood and positive liberty acknowledges the type of autonomy envisaged by Article 12, which requires support for legal capacity. However, the language of 'interdependency' and 'shared personhood' is misplaced and misleading under a strategy that continues to privilege individual autonomy. Under these refashionings of autonomy, rationality is still privileged, the individual is still bounded, and personal space is still sovereign. It is just that the boundary has shifted to include supporters and their rational abilities – but only to the extent that they are instrumental to the adult's decision-making.

Describing all adults with cognitive disability as autonomous in their decision-making stretches the concept beyond its meaningful limits and fails to recognise the residual impacts of impairment. The case of Alex demonstrates that supported decision-making based on recognition of his legal capacity where the decision is

attributed to Alex is fraught with legal and ethical challenges. It can lead to diametrically different outcomes, may lack transparency and accountability, and shroud conflicts of interest and relationships of undue influence. The retention of the privileging of autonomy over other values adheres strongly to the liberal tradition. This leads to a 'fundamental elision' in the prescription for universal legal capacity 'that a finding of mental incapacity will undermine the worth we accord to some persons'[156] who are not autonomous.

Rather than accept the premise that autonomy underpins personhood and worth, a second strategy is to privilege autonomy as one among many valuable social goods – but divorce it from personhood. This allows space for alternative values, inclusive of adults with cognitive disability, to base our recognition of personhood. This strategy refashions autonomy as demanded by Article 12 but recognises that autonomy has its limits and does not have to be privileged above all other values. Autonomy can be conceived of as an achievement or as the development of autonomy competencies, and as inhering within it a demand for choice from a broad range of goods and services.

Both strategies acknowledge the importance of socio-economic supports and services to achieving autonomy, but the first strategy is premised on internal contradictions. One such contradiction was described in Chapter 3 as the 'chicken and egg', or 'initiation', problem whereby supports cannot be claimed without first recognising legal capacity. A second contradiction is that while autonomy as non-interference is privileged, personal supports are promoted and the legal recognition and regulation of support persons are inevitably intrusive. The premises behind what is framed as 'support' and what is 'interference' are inevitably value-laden. A third contradiction is that while the privileging of rationality is disavowed, it is still valued, albeit relocated in the supporter. Under the first strategy, despite attempts to refashion autonomy as interdependence, a bounded concept of the individual is privileged, as is a bounded conception of rights, with Article 12 privileged above all others.

The reformulation of autonomy under the second strategy avoids the above contradictions by adhering less closely to the liberal tradition. It values autonomy as one very important good and Article 12 as one very important right, among others. The theories of autonomy grouped under this second strategy lend themselves to an interpretation of Article 12 that recognises the indivisibility of human rights not just within Article 12 but between Article 12 and other rights in the CRPD. They allow for recognition that the achievement of legal capacity depends on access to a range of socio-economic goods and services. Moreover, the ongoing availability of such goods and services continues to enhance autonomy as meaningful choice. The fulfilment of socio-economic rights is required for the achievement of legal capacity and autonomy. And, depending on an adult's situation in the real world, their immediate need may be the fulfilment of rights beyond Article 12.

[156] Bilchitz (n 26) 417.

In the case of Alex, the appointment of a substitute would not only allow for a decision to be made with and for Alex to have access to basic dental/health care that would ultimately foster social inclusion, but also allow clarity and certainty around who was making the decision and on what basis. In some cases, a supported decision-making model will allow for rights to health and social inclusion to be claimed with certainty and transparency. In other cases, decision-making by a substitute may be an essential tool whereby these other rights and entitlements can be claimed. In many cases, a decision by a substitute may be required to respect the adult's dignity, and it is to the concept of dignity as the basis of personhood that Chapter 5 turns.